3.12

ADULTS

# Obstetrics and Gynaecology Infections

| CHAPTER 3.12 | OBSTETRICS AND GYNAECOLOGY INFECTIONS | | |
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| **Infections and Likely Organisms** | **Suggested Treatment** | | **Notes** |
| | **Preferred** | **Alternative** | |
| **Pelvic Inflammatory Disease** | | | |
| *Chlamydia trachomatis, Neisseria gonorrhoeae, Gardnerella vaginalis, Haemophilus influenzae,* Enteric Gram-negative rods, *Streptococcus agalactiae, Ureaplasma urealyticum, Mycoplasma hominis, Bacteroides sp., Enterobacteriaceae* | **Mild-to-moderate (outpatient regimen):**<br>Ceftriaxone 500 mg IM single dose<br>+<br>Doxycycline 100 mg PO q12H<br>**OR**<br>Azithromycin 1 g PO weekly<br>+<br>Metronidazole 400 mg PO q12H | | • Duration: 14 days.<br>• **Penicillin allergy:** Clindamycin 900 mg IV q8H + Gentamicin 5 mg/kg IV q24H.<br>• Male partners should be contacted and offered screening for chlamydia and gonorrhoea.<br>• Treat all male partners with Azithromycin 1 g PO single dose.<br>• Tubo-ovarian abscess: surgical intervention for source control required.<br>• May need to consider tuberculosis if not responding to standard treatment. |
| | **Moderate-to-severe** | **Moderate-to-severe** | |

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| | **(inpatient regimen):**<br>Ceftriaxone 2 g IV q24H<br>+<br>Doxycycline 100 mg PO q12H<br>+<br>Metronidazole 400 mg PO q12H | **(inpatient regimen):**<br>Ampicillin/Sulbactam 3 g IV q6H<br>+<br>Doxycycline 100 mg PO q12H | |
| **Bacterial Vaginosis** | | | |
| *Gardnerella vaginalis, Mycoplasma hominis, Prevotella spp, Mobiluncus* | Metronidazole 400 mg PO q12H for 5-7 days | Clindamycin 300 mg PO q12H for 7 days<br>**OR**<br>Metronidazole Vaginal Gel LA OD x 5 days* | • Treatment is indicated for symptomatic women, women undergoing gynaecological procedures, and pregnant women.<br>• Avoid alcohol with Metronidazole. No evidence of teratogenicity of use Metronidazole in the first trimester of pregnancy.<br>• Routine screening and treatment of male partners not indicated.<br>• Patients should be advised to avoid vaginal douching and the use of shower gels and strong scented soap.<br>• *Not available in UMMC. |

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| **Vaginal Candidiasis**<br>*Candida albicans* | | | |
| **Uncomplicated**<br>Must fulfil all criteria:<br>• Sporadic or infrequent<br>• Mild-to-moderate symptoms<br>• Likely to be Candida albicans<br>• Non-immunocompromised | **Topical Therapy:**<br>Clotrimazole Pessary 500 mg PV single dose | **Oral Therapy:**<br>Fluconazole 150 mg PO single dose*<br>**OR**<br>Itraconazole 200 mg PO q12H for 1 day* | • Oral therapy is contraindicated in pregnancy.<br>• *Pregnancy risk: Category C. |
| **Complicated**<br>Fulfil any of the criteria:<br>• Recurrent<br>• Severe symptoms<br>• Non-albicans candidiasis<br>• Immunosuppressed | **Severe vaginitis symptoms:**<br>Fluconazole 200 mg q72H for 2 or 3 doses<br>**Recurrent vulvovaginal candidiasis:**<br>Fluconazole 200 mg PO q72H for 3 doses then weekly for 6 months | Clotrimazole Pessary 500 mg once weekly PV for 6 months | |
| **Uncomplicated in** | Clotrimazole Pessary | | |

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| **pregnancy** | 500 mg PV single dose | | |
| **Recurrent episode in pregnancy** | Clotrimazole Pessary 500 mg single dose, repeat 1 dose in a week/after 3-4 days | | |
| **Obstetrics Infections** | | | |
| **Septic abortion/ incomplete/ surgical abortion**<br>*Bacteroides sp (Prevotella bivia)*<br>*Streptococcus sp. (Group A, Group B)*<br>*Enterobacteriaceae*<br>*Chlamydia trachomatis*<br>*Ureaplasma urealyticum* | Ampicillin 2 g IV q4-6H<br>+<br>Gentamicin 5mg/kg IV q24H<br>+<br>Metronidazole 500 mg IV q8H | Cefuroxime 1.5 g IV q8H<br>+<br>Metronidazole 500 mg IV q8H<br>±<br>Doxycycline 100 mg PO q12H*<br><br>**Penicillin Allergy:**<br>Clindamycin 900 mg IV q8H<br>+<br>Gentamicin 5 mg/kg IV q24H | • *Doxycycline is only added when there is positive culture.<br>• Duration: 10-14 days, but 3-7 days for Doxycycline.<br>• Antibiotic should be started as soon as possible when diagnosis is made or at least within 2 hours of the procedure.<br>• IV antibiotics are given until patient has improved and afebrile for 48 hours and followed by oral antibiotics. |
| **Intra-partum antibiotic** | Benzylpenicillin 5MU IV | Ampicillin 2 g IV single | • Prophylaxis begins at hospital admission for labour or |

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| **prophylaxis (IAP) for *Group B Streptococcus* (GBS)**<br>Indications for IAP:<br>• Previous infant with invasive GBS disease<br>• Preterm labour<br>• GBS carriage in previous pregnancy<br>• PPROM with known GBS carrier<br>• GBS carriage in current pregnancy | single dose, then 2.5–3MU IV q4H until delivery | dose, then 1 g IV q4H until delivery<br><br>**Penicillin Allergy (Mild/ Low risk of anaphylaxis):**<br>Cefazolin 2 g IV single dose, then 1 g IV q8H until delivery<br>**OR**<br>Cefuroxime 1.5 g IV single dose and 750 mg IV q8H until delivery<br><br>**Penicillin Allergy (Severe/ High risk of anaphylaxis)***:<br>Clindamycin 900 mg IV q8H until delivery<br>**OR**<br>Vancomycin 15-20 mg/kg IV q8-12H until delivery | rupture of membrane and continues every four hours until the infant is delivered.<br>• Treatment is NOT INDICATED if caesarean section is performed before onset of labour with intact membrane (please use standard surgical prophylaxis).<br>• Antenatal treatment is recommended for GBS bacteriuria and for symptomatic patients. |

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| **Preterm Premature Rupture of Membranes (PPROM)** | **Non-GBS carrier:**<br>Erythromycin Ethyl Succinate 400 mg PO q12H for 7-10 days<br><br>**GBS carrier:**<br>Ampicillin 2 g IV q6H for 48 hours<br>Followed by,<br>Erythromycin Ethyl Succinate 400 mg PO q12H for 7-10 days | **Penicillin Allergy (Mild/ Low risk of anaphylaxis):**<br>Cefazolin 2 g IV single dose, then 1 g IV q8H until delivery<br>**OR**<br>Cefuroxime 1.5 g IV single dose and 750 mg IV q8H until delivery<br><br>**Penicillin Allergy (Severe/ High risk of anaphylaxis)***:<br>Clindamycin 900 mg IV q8H until delivery | • Review culture to treat accordingly. |
| **Chorioamnionitis** | Cefuroxime 1.5 g IV single dose then 750 mg IV q8H<br>+<br>Metronidazole 500 mg IV q8H | **Penicillin Allergy (Mild/ Low risk of anaphylaxis):**<br>Cefazolin 2 g IV single dose, then 1 g IV q8H<br>+ | • Antibiotic regimen is continued postpartum until patient achieves good clinical response.<br>• Source control: Removal of infected product of conception. |

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| | | Metronidazole 500 mg IV q8H<br><br>**Penicillin Allergy (Severe/ High risk of anaphylaxis)***:<br>Clindamycin 900 mg IV q8H | |
| **Cervicitis** | Azithromycin 1 g PO single dose | Doxycycline 100 mg PO q12H for 7 days | |
| **Postpartum mastitis**<br>*Staphylococcus aureus (*MSSA*)*<br>*Streptococcus pyogenes (*Grp A, B*)*<br>*Escherichia coli*<br>*Bacteroides sp*<br>*Corynebacterium sp*<br>CoNS | **Outpatient Therapy:**<br>Cloxacillin 500 mg PO q6H for 7 days<br>**OR**<br>Co-amoxiclav 625 mg PO q12H for 7 days<br><br>**Inpatient Therapy:**<br>Cloxacillin 2 g IV q6H | **Penicillin Allergy (Mild/ Low risk of anaphylaxis):**<br>Cefazolin 2 g IV single dose, then 1 g IV q8H<br><br>**Penicillin Allergy (Severe/ High risk of anaphylaxis)***:<br>Clindamycin 900 mg IV q8H | • Duration of therapy for 5-7 days may be adequate but if poor response, consider extending to 10-14 days.<br>• Milk culture for less severe infection.<br>• If severe infection (hemodynamic instability), blood culture is required. |

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